

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Multicentre, Phase II, Single-Arm, Interventional Study of Neoadjuvant Durvalumab and Platinum-based Chemotherapy (CT), Followed by Either Surgery and Adjuvant Durvalumab or Chemoradiotherapy (CRT) and Consolidation Durvalumab, in Participants With Resectable or Borderline Resectable Stage IIB-IIIB Non-small Cell Lung Cancer (NSCLC)
Short Title:	Study to Assess Neoadjuvant Durvalumab (D) and Platinum-Based Chemotherapy (CT), Followed by Either Surgery and Adjuvant D or CRT and Consolidation D, in Resectable or Borderline Resectable Stage IIB-IIIB NSCLC (MDT-BRIDGE)
Protocol Number:	1088127532596554058
NCT Number:	NCT05925530
Sponsor Name:	AstraZeneca
Investigational Product:	durvalumab
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Resection rate: Resection rate is defined as the proportion of all participants who underwent definitive surgery. Participants who undergo (ie, start) surgery with the goal of complete tumour resection will be counted as meeting this endpoint.
Secondary Obj.:	1. Resection rate 2. R0, R1, R2 resection rates 3. Pathological complete response (pCR)

2.2 Background & Rationale

The purpose of this study is to assess efficacy and safety of neoadjuvant durvalumab in combination with platinum-based chemotherapy (CT) given as initial therapy after cancer diagnosis followed by either surgery and adjuvant durvalumab or chemoradiotherapy (CRT) and consolidation durvalumab given alone as further therapy in participants with resectable and borderline resectable stage IIB-IIIB NSCLC.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: ACTIVE_NOT_RECRUITING
Target N:	142

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

* Deemed resectable or borderline resectable at baseline, confirmed by MDT evaluation at diagnosis.

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- * Previously untreated and pathologically confirmed Stage IIB to select \[i.e.N2\] Stage IIIB by AJCC v8.
- * Nodal status confirmed with whole body FDG-PET and biopsy via endobronchial ultrasound, mediastinoscopy, or thoracoscopy.
- * Mandatory brain MRI.
- * EGFR and ALK wild-type.
- * Medically operable: adequate cardiac and lung function to undergo resection.
- * Participant must be ≥ 18 years, at the time of screening.
- * Histologically or cytologically documented NSCLC.
- * Minimum life expectancy of 12 weeks.
- * Minimum body weight of 30 kg.
- * Male and female participants must be willing to use acceptable methods of contraception.
- * Female participants of childbearing potential must have negative pregnancy test.

4.2 Exclusion Criteria

- * Unresectable NSCLC confirmed by MDT evaluation at baseline
- * Stage IIIC patients
- * Participants whose planned surgery at enrollment is a wedge resection
- * Known EGFR mutation or ALK translocation
- * Participants contraindicated for surgical intervention due to comorbid conditions
- * Participants who are allergic to study intervention.
- * Participants with more than one primary tumour.
- * Known active hepatitis infection, positive HCV antibody, HBsAg or HBV core antibody (anti-HBc), at screening.
- * Female participants who are pregnant or breastfeeding.
- * Judgement by the investigator that the participant should not participate in the study.
- * Previously infected or tested positive for human immunodeficiency virus.

11. Study Identifiers & Governance

Primary ID:	1088127532596554058
Registry Number:	NCT05925530
Sponsor:	AstraZeneca
Study Title:	Study to Assess Neoadjuvant Durvalumab (D) and Platinum-Based Chemotherapy (CT), Followed by Either Surgery and Adjuvant D or CRT and Consolidation D, in Resectable or Borderline Resectable Stage IIB-IIIB NSCLC (MDT-BRIDGE)

15. Sign-off & Approval

Principal Investigator: _____	Date: _____
Clinical Operations Lead: _____	Date: _____
Lead Statistician: _____	Date: _____